

Sores - Female (Child)

Algorithm 1280 | Child (0-17 years) | Gender at birth: Female

Definition

- Qatar-localized UAT-only adult and pediatric pathway for a nonspecific skin sore, screening for sepsis, rapidly spreading infection, high-risk sites, diabetes/immune risk and safeguarding; not approved for production.
- Child patient.
- Female.

Initial assessment

1. [LOCATION] Where is the sore?
2. [DURATION] How long has it been present?
3. [YES_NO] Any fever or feeling generally unwell?
4. [OPEN_TEXT] What is the exact age; is the patient pregnant; is the sore near an eye, face, genital area or joint; and are there diabetes, poor circulation, immune suppression, pressure injury, rapidly spreading redness, severe pain, recurrent lesions or possible neglect/deliberate injury?
5. [CAREGIVER_OBSERVATION] What has a safe parent or caregiver observed about alertness, breathing, pain, swelling, fever, eating or drinking, sleep, play, and normal activity?

Triage and redirect questions

1. Redirect → Boil (Skin Abscess)

Is this primarily a painful pus-filled lump or abscess rather than an open or non-healing sore?

A painful pus-filled lump or abscess belongs in the Boil (Skin Abscess) pathway.

1. Disposition level 70

Is the skin around the sore hot, painful, and swollen, is there a high temperature or feeling hot/cold/shivery, or a weakened immune system?

NHS.UK guidance for skin infections lists these as reasons for urgent clinical review.

1. Disposition level 50

Has the sore lasted 2 weeks or more without improving, or does it keep recurring?

A sore that won't heal after a couple of weeks needs primary-care review to check for an underlying cause.

1. Disposition level 70

After higher-risk features are excluded, is there still a new or unexplained skin sore?

Because this is a nonspecific synthesized pathway, in-person assessment is required before assuming a benign cause or recommending self-care.

Advice

Urgent sore review

- Arrange same-day in-person review through the locally approved Qatar UAT pathway. Do not squeeze, lance or share dressings, and do not start leftover antibiotics.
- Call Back If: spreading redness or severe pain; fever, drowsiness or rapid deterioration.

Persistent sore follow-up

- Arrange in-person primary-care review for a sore that will not heal or keeps recurring. Document diabetes, pressure, vascular, immune and safeguarding risks.
- Call Back If: worsening pain, redness, discharge or systemic illness.

In-person review for a new sore

- Keep the area clean and lightly covered, avoid picking or applying unverified products, and arrange in-person review through the Qatar pathway approved for UAT. Medication advice requires patient-specific checks.
- Call Back If: spreading redness, severe pain or discharge; fever or deterioration develops.

First aid

References

- NHS.UK, "Boils", <https://www.nhs.uk/conditions/boils/> (page last reviewed 20 June 2023)

Provenance

Shaped to mirror the licensed After-Hours Telehealth Triage Guidelines database structure. This is IST open-source guideline content derived from cited public sources; it is not licensed STCC content. Question.DispositionLevel=null marks a See More Appropriate Guideline redirect. Demographic variants contain only source-supported differences; identical thresholds are documented rather than fabricated. Local destinations and TelemedicineEligible values require IST clinical governance approval. UAT DATA ONLY - NOT FOR REAL-PATIENT CARE OR PRODUCTION USE.

Manifest hash: 40173e8f0e99dea5a67aef37fb812610aa386cc754d7231eed0b3a0ab169c0cb | Status: BLOCKED_BY_RESEARCH_GAP